



SUSPECTED ADVERSE DRUG REACTION REPORTING FORM

For **VOLUNTARY** reporting of ADRs by Healthcare Professionals

INDIAN PHARMACOPOEIA COMMISSION (National Coordination Centre-Pharmacovigilance Programme of India)

Ministry of Health & Family Welfare, Government of India, Sector-23, Raj Nagar, Ghaziabad-201002

PvPI Helpline (Toll Free) : 1800-180-3024 (9:00 AM to 5:30 PM, Monday-Friday)

Initial Case ☐		Follow-up Case ☐		FOR AMC / NCC USE ONLY							
A. PATIENT INFORMATION *				Reg. No. / IPD No. / OPD No. / CR No. :							
1. Patient Initials: A.K.		2. Age or date of birth: 34 years		AMC Report No. :							
3. Gender: M ☐ F ☒ Other ☐		4. Weight (in Kg.) 62		Worldwide Unique No. :							
B. SUSPECTED ADVERSE REACTION *				12. Relevant investigations with dates :							
5. Event / Reaction start date (dd/mm/yyyy)		03/03/2026		13. Relevant medical / medication history (e.g. allergies, pregnancy, addiction, hepatic, renal dysfunction etc.)							
6. Event / Reaction stop date (dd/mm/yyyy)		06/03/2026									
7. Describe Event/Reaction management with details, if any Generalized pruritic rash and itching developed after amoxicillin. Amoxicillin was stopped immediately. Cetirizine 10 mg orally daily was given for symptomatic relief. The reaction improved after withdrawal of the suspected medicine and resolved by 06/03/2026.											
				14. Seriousness of the reaction : No ☐ if Yes ☐ (please tick anyone) ☐ Death (dd/mm/yyyy) ☐ Congenital-anomaly ☐ Life threatening ☐ Disability ☐ Hospitalization-Initial/Prolonged ☐ Other Medically important							
				15. Outcome: ☒ Recovered ☐ Recovering ☐ Not Recovered ☐ Fatal ☐ Recovered with sequelae ☐ Unknown							
C. SUSPECTED MEDICATION(S) *											
S. No.	8. Name (Brand / Generic)	Manufacturer (if known)	Batch No. / Lot No.	Expiry Date (if known)	Dose	Route	Frequency	Therapy Dates		Indication	Causality Assessment
								Date Started	Date Stopped		
i	Amoxicillin	-----	-----	-----	500 mg	PO	TID	01/03/2026	03/03/2026	bacterial sinusitis	Probable/Likely
ii											
iii											
iv [#]											
9. Action taken after reaction (please tick)								10. Reaction reappeared after reintroduction of suspected medication (please tick)			
S. No. as per C	Drug withdrawn	Dose increased	Dose reduced	Dose not changed	Not applicable	Unknown	Yes	No	Effect unknown	Dose (if re-introduced)	
i	Drug withdrawn							NO reappearance			
ii											
iii											
iv											
11. Concomitant medical product including self-medication and herbal remedies with therapy dates (Exclude those used to treat reaction)											
S. No.	Name (Brand / Generic)	Dose	Route	Frequency (OD, BD, etc.)	Therapy Dates		Indication				
					Date Started	Date Stopped					
i	None Reported										
ii											
iii [#]											
Additional Information : Patient identified only by initials for confidentiality. NO rechallenge was performed. Causality assessment: Probable/Likely and important after withdrawal.						D. REPORTER DETAILS *					
						16. Name & Address : Dr. Meera Sharma India Pin : 0000000 Email : meerasharma@example.com Contact No- : 00000 00000 Occupation : Clinical Pharmacist Signature : M. Sharam					
						17. Date of this report (dd/mm/yyyy) :					
Signature and Name of Receiving Personnel : _____											
Confidentiality : The patient's identity is held in strict confidence and protected to the fullest extent. Submission of a report does not constitute an admission that medical personnel or manufacturer or the product caused or contributed to the reaction. Submission of an ADR report does not have any legal implication on the reporter.											

[#] Use separate page for more information

* Mandatory Fields for suspected ADR Reporting Form

ADVICE ABOUT REPORTING

A. What to report?

All adverse events should be reported

Report non-serious, known or unknown, frequent or rare adverse drug reactions due to Medicines, Vaccines & Herbal Products.

Report every serious adverse drug reactions. A reaction is serious when the patient outcome is :

- Death
- Life-threatening
- Hospitalization (initial or prolonged)
- Disability (significant, persistent or permanent)
- Congenital anomaly
- Report intervention to prevent permanent impairment or damage

NOTE : Serious/Adverse Event following immunization can also be reported in Serious AEFI case Notification Form available on <http://www.ipc.gov.in>

B. Who can report?

All healthcare professionals (Clinicians, Dentists, Pharmacists and Nurse etc.) can report adverse drug reactions

C. Where to report?

Duly filled in Suspected Adverse Drug Reaction Reporting Form can be sent to the nearest Adverse Drug Reaction Monitoring Centre (AMC) or directly to the National Coordination Centre (NCC) for PvPI.

Call on Helpline (Toll Free) 1800 180 3024 to report ADRs or directly mail this filled form to pvpi.ipc@gov.in

A list of nationwide AMCs is available at : <http://www.ipc.gov.in>, http://www.ipc.gov.in/PvPI/pv_home.html

D.What happens to the submitted information?

- Information provided in this form is handled in strict confidence. The causality assessment is carried out at AMCs by using WHO-UMC scale. The analyzed forms are forwarded to the NCC-PvPI through ADR database. Finally the data is analyzed and forwarded to the Global Pharmacovigilance Database managed by WHO Uppsala Monitoring Centre in Sweden.
- The reports are periodically reviewed by the NCC-PvPI. The information generated on the basis of these reports helps in continuous assessment of the benefit-risk ratio of medicines.
- The Signal Review Panel of PvPI reviews the data and suggests any interventions that may be required.

E. Mandatory fields for suspected ADR Reporting Form (*)

Patient initials, age at onset of reaction, reaction term(s), date of onset of reaction, suspected medication(s) & reporter information.

For Adverse Drug Reaction Reporting Tools

- E-mail : pvpi.ipc@gov.in
- PvPI Helpline (Toll Free) : 1800 180 3024 (9:00 AM to 5:30 PM, Monday-Friday)
- ADR Mobile App : "ADRPvPI"